Southern Well

OF SOUTH MISSISSIPPI

Standard Operating Procedures

Case Management Services
Revision 2

Supporting Healing. Strengthening Families.

Service area: Forrest, Lamar, Jones and Stone Counties, Mississippi
601-554-6067
Internal use only. Confidential. Draft for owner review | October 2026

Non-clinical care navigation, case management, wellness support, advocacy and resource coordination

Document Control

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I. Purpose and How to Use This Guide

SouthernWell Case Management Services provides private-pay, **non-clinical** care navigation, case management, wellness support, advocacy and resource coordination for seniors, individuals with developmental needs and their families. This guide sets one consistent, safe way of doing the work so every client receives professional service.

- **Order of authority.** Applicable law and regulation come first. A licensed provider's written instructions come next. This guide follows. If they conflict, follow the higher authority and tell the owner.
- **Sequential execution.** Follow each procedure in order. When unsure, stop and use SOP-10 (emergency and escalation) or ask the owner.
- **Documentation standard.** Every client interaction is documented (SOP-12). If it was not written down, it did not happen.
- **Annual governance.** Review this guide at least yearly and log every change in the Revision Log (Appendix C).

Key definitions

Term	Meaning in this guide
Non-clinical	Coordination, navigation, reminders, observation and advocacy. Excludes diagnosing, assessing, treating, counseling, or advising on medications, diet or exercise.
Client	The person receiving SouthernWell services. Never "patient."
Authorized representative	A person with legal authority to act for the client (power of attorney, healthcare representative or guardian), documented in the file.
Business day	Monday to Friday, excluding holidays.
Gate 1 / Gate 2	Gate 1 = Enrollment Complete (documents and payment). Gate 2 = Ready to Start (screening, welcome call, emergency contacts). See SOP-03.
Provider-directed parameters	Instructions about when to call the provider, written by the client's provider. SouthernWell never creates thresholds or targets.

Before operational deployment

- Obtain legal review of scope, consent, release, service agreement and consulting agreements.
- Confirm insurance coverage (professional liability, general liability, commercial auto if transporting clients) for every service listed.
- Complete the open items in Appendix D.

II. Programs, Prices and Service Area

The current **Price Sheet** is the controlling source for prices. This table summarizes it; if they differ, the Price Sheet governs and this guide must be corrected.

Program	Who and what it covers	Monthly price
SouthernWell Essentials	Initial wellness and support assessment (non-clinical), personalized action plan, resource identification and referrals, care navigation support, one monthly check-in, email support.	$150
SouthernWell Signature (most popular)	Non-clinical case management, care navigation and coordination, personalized care planning, resource referrals, two monthly check-ins, wellness accountability support, family communication support, priority email and phone support.	$225
SouthernWell Concierge	Comprehensive care navigation and non-clinical case management, care planning, wellness accountability, weekly check-ins, family coordination, priority communication, advocacy and resource coordination, customized wellness action plans.	$300
Add-ons	Medication Concierge† (reminders, medication list organization, refill and pharmacy coordination; observation only); Steady Wellness™ (wellness tracking support, accountability check-ins, progress reporting); Integrated Wellness Circle™ (family wellness meetings, caregiver support, community resource connections, care planning support).	$25† / $50 / $35
Medication Concierge plans†	Clients living independently. Observation-only support while the client fills their own organizer, SouthernWell pill organizer included. One weekly visit (Magnolia Medication Plan) or two weekly visits (Signature Medication Concierge).	$125 / $150
Telephonic Services Program	Clients 75 to 100 miles away. Package 1: one scheduled in-person visit per month, four telephonic check-ins, provider coordination as needed. Package 2: everything in Package 1 plus two visits per month and unlimited telephonic check-ins.	$195 / $225
Neighborly Rate (sliding scale)	Reviewed case by case on income, hardship, caregiver burden and special circumstances. Essentials $100 to $125; Signature $150 to $200; Concierge $225 to $275. Limited availability.	Per approval
Pay-as-you-go components	Front Porch Flex in-home check-ins; Porch-to-Provider appointment companion; Southern Vitals self-measured readings log; Well Connected resource navigation. Hourly range $25 to $60.	Per Price Sheet

† Medication Concierge appears both as a $25 add-on and as standalone plans at $125 and $150. Confirm which applies to which client type (Appendix D, item 3).

Service area and contact

- **In-person service area:** Forrest, Lamar, Jones and Stone Counties, Mississippi. (The September draft also listed Harrison, Jackson and Pearl River; these are removed.)
- **Telephonic Services Program:** clients 75 to 100 miles from the service area.
- **Contact standard:** phone 601-554-6067 on all materials. Payment: private pay only; accepted methods are listed in SOP-14. LCSW services are billed separately by the LCSW.

III. Roles and Responsibilities

Role	Operational scope
Owner and Care Coordinator	Manages onboarding, check-ins, home visits, documentation and billing. Makes escalation decisions and maintains these standards. Holds the Client ID register and approves sliding scale and exceptions.
Consulting LCSW (independent)	Completes psychosocial and clinical intakes under her own license. Billed separately to clients. No referral fees or compensation of any kind between SouthernWell and the LCSW (SOP-09).
Consulting RN Reviewer (independent)	Advisory only. Audits scripts and documentation samples monthly and after a status change to confirm non-clinical scope. Does not deliver or direct client care.
Support staff (when hired)	Background check, CPR/First Aid and SOP training completed and documented before any solo service delivery. Works only within SOP-01.
Client and authorized representative	Provides accurate information, follows provider instructions, keeps appointments and notifies SouthernWell of changes in contact information, health status or address.

SOP-01 Scope of Practice and Approved Wording

Purpose: Keep every service within non-clinical care coordination.

Approved scope	Prohibited scope
• Check in regularly and record observations in the client's own words • Remind clients of appointments, refills and provider directions • Keep logs of self-measured readings to share with providers • Coordinate transportation, resources and administrative paperwork • Observe (without touching) while the client fills their own pill organizer • Update designated family with signed consent	• Diagnosing, assessing, treating or evaluating conditions or readings • Setting clinical parameters, thresholds or target numbers • Selecting, counting, administering, verifying or changing medications • Medical, dietary, exercise or treatment advice • Counseling or psychotherapy • Handling client money or signing medical consents

Approved wording standard

Use this	Never use this
Medication reminders and organization support	Medication management, administration or clinical oversight
Log self-measured readings for provider review	Monitor vital signs or evaluate clinical trends
Check in, coordinate care, support	Assess, evaluate, treat, manage clinically
Care Plan (non-clinical) or Family Support Plan	Plan of care, clinical treatment plan
Client and family	Patient
Non-clinical care coordination	Behavioral health treatment, nursing care, clinical therapy
Screening of social needs (SDOH)	Psychosocial or clinical assessment (reserved for the LCSW)

When a client asks a clinical question

Say: "That is a question for your provider. I can help you write it down and bring it to your appointment, or help you call the office." Do not interpret, reassure or advise. If symptoms sound urgent, go to SOP-10.

SOP-02 Inquiry, Eligibility and Fit Decision

Purpose: Decide, in writing and quickly, whether SouthernWell can serve a caller safely.

1. **Safety screen first.** Ask whether anyone is in immediate danger or having a medical emergency. If yes, direct to 911 and document. Stop intake.
2. **Initial contact.** Assign a Client ID (SW-YY-###). Record caller name and relationship, client name, county, contact details, call times, language or communication needs, decision maker, and referral source.
3. **Four-test fit decision (by next business day):** Geography (service area or Telephonic Program range); Scope (needs are non-clinical); Capacity (caseload below the owner-set cap, currently planned at 15 to 20 active clients); Safety and conduct (no known risk that cannot be managed under SOP-05 and SOP-15).
4. **Record one outcome:** Accept, Waitlist, Refer out or Decline, with the reason. Communicate it by the next business day. Give at least two community resources when referring out or declining.
5. **Program fit.** For Accept, present the suited program and Price Sheet, and send the enrollment packet (Workflow Manual Step 3).

SOP-03 Intake and Enrollment

Purpose: Bring each client on board with a single, auditable path. The Workflow Manual holds the detailed steps and scripts; this SOP sets the rules.

Phase	Workflow steps	Rule
Inquiry and fit	1 to 2	Safety screen, Client ID, four-test fit decision, documented outcome.
Packet	3 to 4	Packet sent in one send; Completeness Review; one consolidated request for missing items; reminders Day 2, 5 and 8; close at Day 10 if not returned.
Consents and payment	5 to 6	Authority to sign verified; HIPAA authorization, Consent and Enrollment Agreement, Service Agreement and Release signed; payment confirmed. **Gate 1: Enrollment Complete.**
Onboarding	7 to 9	SDOH screening and welcome call within 3 business days of Gate 1; Emergency Contact Sheet with missed check-in and welfare check authorization. **Gate 2: Ready to Start.** Care Plan within 7 calendar days of Gate 1.
Active	10	Services begin only after Gate 2. Day-30 review completed and logged.

Standing rules

- No service, provider call or family update before the signed documents covering it are on file.
- **Authority to sign:** accept signatures only from the client or an authorized representative. File a copy of the authorizing document. If capacity or authority is in doubt, pause and consult the owner or legal counsel.
- Required before Gate 1: Service Agreement and Privacy Notice; HIPAA Privacy and Communication Authorization; Consent and Enrollment Agreement; Information Release (provider, therapist, pharmacist; family if involved); payment authorization; Observation-Only Medication Consent if the Medication Concierge applies.
- The “Psychosocial Intake and Needs Assessment” in the earlier draft is retired. SouthernWell performs a non-clinical intake review and SDOH screening. Psychosocial and clinical assessment belongs to the independent LCSW.
- Maintain one client record per Client ID. Releases carry expiration dates tracked in the Google Sheet.

SOP-04 Telephonic Check-In Calls

Purpose: Provide consistent, documented, non-clinical check-ins.

1. **Prepare:** review prior Call Notes, the Care Plan and open tasks before dialing.
2. **Verify identity:** confirm two identifiers (name plus date of birth or address) before discussing anything.
3. **Engage:** use the client’s preferred call times. Record updates in the client’s exact words.
4. **Coordinate:** identify barriers (transportation, refills, resources) and help with non-clinical navigation.
5. **Document** the Call Note in STAR format the same business day and no later than 24 hours.
6. **Emergency:** if the client reports severe distress (chest pain, trouble breathing, stroke signs, sudden confusion, thoughts of self-harm), go to SOP-10 immediately.
7. **Voicemail:** leave only staff name and 601-554-6067 (SOP-13).

SOP-05 In-Home Visits and Home Safety Check-In

Purpose: Observe and report, safely.

1. **Before:** confirm the visit by phone and log destination, arrival time and expected end time. Share the schedule with a designated check-in contact.
2. **Safety check:** visually observe walkways, lighting, telephone access, emergency contact placement, smoke detectors, food storage and room temperature. Record what you see, not what you conclude.

3. **Boundaries:** give observational notes to the client or family. Do not repair fixtures, adjust medical equipment or provide personal care.
4. **Staff safety:** if the environment presents immediate hazards, exit safely and notify the owner. Text the check-in contact on arrival and departure.
5. **After:** complete the In-Home Visit Note (STAR) the same business day.

SOP-06 Medication Concierge Visits

Purpose: Observation-only support for clients who fill their own organizer.

1. Verify the signed Observation-Only Medication Consent. Provide the complimentary SouthernWell pill organizer.
2. The client (or a designated family member) fills the organizer from pharmacy-labeled packaging and provider orders.
3. Staff remain present only to observe and encourage. Staff never touch, count, sort or hand over medications.
4. Document factual observations, for example: “Client filled own organizer; staff reminded client to follow original pharmacy labels.”
5. **If the client asks staff to help with the pills,** or cannot fill the organizer: decline, document, suggest the client contact the pharmacist or provider, and notify the owner and (with consent) the family. Do not substitute.
6. **If something looks inconsistent** (missed doses reported, a label that does not match what the client says): do not interpret. Encourage the client to call the pharmacist or provider and document exactly what was said and observed.

SOP-07 SteadyCare Chronic Condition Support

Purpose: Support routine and self-management for adults with hypertension, COPD, type 2 diabetes or hyperlipidemia, using provider recommendations only.

1. **At enrollment,** ask the client to bring the provider’s written instructions on when to call the office or seek urgent care. File them. SouthernWell does not write or change them.
2. Hold weekly check-ins focused on routine adherence and the client’s own self-measured logs.
3. Log readings exactly as the client gives them. Respond neutrally: “Thank you for sharing.” Do not label numbers high, low or stable.
4. If the client reports a reading outside the provider’s written instructions, or asks if a number is okay: “Your provider’s instructions say when to call. Would you like help calling the office now?” Document.
5. If the client reports emergency symptoms, go to SOP-10.
6. Send the compiled log to the managing provider only with written authorization (Monthly Provider Update).

SOP-08 Appointment Companion (Porch-to-Provider)

1. Align the appointment calendar and arrange transportation (Transportation and Appointment Request form).
2. Accompany the client; help with check-in paperwork and comfort.
3. If attending the visit at the client’s request, take written notes for client and family review.
4. Do not give clinical opinions, interpret medical advice or sign medical consent forms.

SOP-09 Family Support Program and LCSW Collaboration

1. Keep a clear line between SouthernWell care coordination and the independent LCSW’s clinical care.
2. Execute mutual releases before exchanging coordination notes.
3. Use LCSW recommendations to build a Family Support Plan limited to non-medical tasks, family roles and community support.
4. Keep administrative and client records completely separate. No referral fee, credit, gift or other compensation passes in either direction between SouthernWell and the LCSW.
5. Licensed professionals (LCSW, RN reviewer, providers) are excluded from Community Referral Program credits unless legal counsel confirms otherwise (Appendix D, item 2).

SOP-10 Emergency, Crisis and Missed Check-In Response

Situation	Action protocol
Acute medical emergency (chest pain, severe shortness of breath, stroke symptoms, loss of consciousness)	Call 911 immediately and stay on the line if possible. After emergency services are dispatched, contact the emergency contact. Document and file an Incident Report within 24 hours.
Behavioral health or suicide crisis	Call 911 for immediate danger, or 988 Suicide and Crisis Lifeline. Stay with the client on the line. Notify emergency contacts and the consulting LCSW as consent allows. Incident Report within 24 hours.
Missed scheduled check-in	1) Call again twice at 15-minute intervals (three attempts total). 2) Text if the client has consented. 3) Call the first emergency contact. 4) If no contact is made and a safety risk is suspected, or the client has not been reached within 2 hours of the scheduled time, request a welfare check from local law enforcement as authorized on the Emergency Contact Sheet. 5) Document each attempt with time and result.
Client fall	Do not lift or move the client. Call 911 if injured or unable to rise comfortably. Notify emergency contact. Incident Report.
Severe weather, power or phone outage	Use the Emergency Contact Sheet to reach clients flagged as high-need first. Document each contact.
Unable to decide	Treat as an emergency. Call 911 or the owner. Document the decision and reason.

After any emergency: notify the consulting RN reviewer at the next monthly review (or immediately if it changes the client's status), and review the Care Plan.

SOP-11 Abuse, Neglect, Exploitation and Incident Reporting

- **Report promptly.** Report any observed or reported sign of abuse, neglect, physical harm or financial exploitation of an adult to Mississippi Adult Protective Services (1-844-437-6282). Do not investigate and do not wait for proof.
- **Immediate danger:** call 911 first.
- **Objective records only.** Write facts and direct quotes, not opinions or unverified accusations. File an internal Incident Report within 24 hours.
- **Confidentiality:** share only what is needed to make the report; do not discuss the report with the alleged person responsible.

SOP-12 Documentation and Record Keeping

- Log every call, visit and coordination task the **same business day**, and no later than 24 hours. Use the STAR method (Situation, Task, Action, Result).
- Record factual observations and direct client quotes. Avoid clinical impression terms and diagnostic labels.
- **Corrections:** single strikethrough, initial and date. Never overwrite or delete history. In electronic records, add a dated correction entry.
- **Late entries** are labeled “Late entry,” dated with the actual entry date and state why.
- Store electronic records in encrypted, access-controlled software. Back up regularly. No client information on personal messaging apps or unsecured notes.
- Each client has one file organized by Client ID in the order of the Master Forms Checklist.
- Retention and destruction periods are set after legal review (Appendix D, item 5) and recorded here once confirmed.
- Clients may request a copy of their records in writing; respond within the period set by law and the Service Agreement.

SOP-13 Privacy, Consent and Communication

- Disclose information only to people named on the signed Release of Information or Family Communication Authorization, and only the minimum necessary.
- Verify identity before discussing client details by phone (SOP-04).
- **Voicemails and texts:** staff name and 601-554-6067 only, with no care details. Use text or email for scheduling only unless the client has agreed in writing to a specific channel.
- Keep devices password-protected and locked. Do not photograph clients, medications or documents on personal devices unless the record system requires it and the client consents.
- **Suspected privacy incident** (lost device, misdirected message, wrong recipient): tell the owner the same day, log what happened, take steps to limit harm and follow notification duties set by law.
- Track Release expiration dates. Renew or stop disclosure when a release expires or is revoked.

SOP-14 Billing, Payments and Sliding Scale

- SouthernWell is private-pay only. It does not bill Medicare, Medicaid or private health insurance.
- Written Service Agreement and fee schedule before services begin. Payment is due monthly before services begin.
- Accepted methods: credit or debit card, ACH bank draft, Cash App, PayPal, Zelle or invoice. Use a payment-processor link; do not record full card or bank numbers on paper forms or the Sheet.
- **Neighborly Rate** is approved by the owner on a Sliding Scale Request and reviewed every six months.
- **Late or missed payment (proposed):** reminder on the due date; second notice on Day 5; written notice of pause on Day 10; services resume when paid. Hardship is handled through the Neighborly Rate. Confirm in Appendix D, item 4.
- Referral credits are applied to the referrer’s next month and recorded. Cancellation and refund terms follow the signed Service Agreement.
- Staff may not accept gifts or cash, handle client bank accounts or make purchases with client funds.

SOP-15 Scheduling, Travel, Service Area and Continuity

- Coordinate in-person activities only in Forrest, Lamar, Jones and Stone Counties. Clients 75 to 100 miles away are served through the Telephonic Services Program.
- Log exact business mileage for field work and companion travel.
- Transporting clients requires active commercial auto coverage that applies to client transport.
- **Continuity plan:** name a backup contact and tell every client in the Handbook who will reach them if the Care Coordinator is unavailable. For planned absences, give clients 7 days' notice and schedule coverage for high-need clients first. Maintain an up-to-date list of active clients and emergency contacts that a designee can access.
- **Capacity:** the owner sets the active caseload cap; at cap, new clients go on the waitlist (SOP-02).

SOP-16 Quality Review, Training and Updates

- **Monthly:** consulting RN reviewer audits sample documentation and scripts for non-clinical scope.
- **Quarterly:** audit client files for signed consents, authority to sign, current releases, accurate pricing and timely documentation.
- **Annual:** CPR/First Aid, privacy, scope boundaries and reporting duties training. Log completion.
- **Intake measures (review quarterly):** days from inquiry to packet sent; packet return rate; percent of files complete at Gate 1; days from inquiry to start; percent of notes filed on time; number of waitlisted and declined inquiries with reasons.
- Log every change to this guide in Appendix C.

SOP-17 Client Rights, Concerns and Complaints (new)

Purpose: Give clients a clear, safe way to raise concerns.

1. The Client Handbook states client rights: respectful service, privacy, understanding of fees, the right to ask questions, to decline or end services and to raise concerns without retaliation.
2. Record any concern or complaint (written or verbal) with date and the client's words.
3. Acknowledge within 2 business days. Aim to resolve or give a written response within 10 business days.
4. Log the outcome and any change made. Review patterns at the quarterly audit.

SOP-18 Discharge, Transition and Record Closure (new)

Purpose: End services safely, fairly and in writing.

1. **Reasons:** client request, move outside the service area, needs beyond non-clinical scope, non-payment, safety or conduct concerns, or death.
2. Give written notice (except death or immediate safety risk) with the reason, the last service date and community resources. Offer a transition call.
3. With authorization, notify the provider and family. Hand off any open tasks and return client items.
4. Complete final documentation and close the Care Coordination Tasks Log. Record the closure reason and date in the Sheet.
5. Stop disclosures when releases end; retain and later destroy the record per SOP-12.

Appendix A: Forms Index

Form	Used at	Status
Inquiry and Discovery Call Notes	Workflow Step 1	Built; update fields (see Workflow Manual, Appendix A)
Scope and Eligibility Checklist	Step 2	Built; add four-test decision
New Client Intake and Enrollment Packet (8 sections)	Steps 3 to 6	In use
Service Agreement and Privacy Notice	Step 5	Built; add signer authority section; legal review pending
HIPAA Privacy and Communication Authorization	Step 5	In use; legal review pending
Release of Information (provider, therapist, pharmacist)	Step 5	Built; legal review pending
Family Communication Authorization	Step 5	Built; legal review pending
LCSW Referral Acknowledgment	Step 5	Built
Medication Concierge Enrollment Packet and Observation-Only Consent	Step 5	In use
Sliding Scale Request	Step 6	Built
SDOH Screening Form	Step 7	Built
Client Handbook Acknowledgment and Emergency Contact Sheet	Step 8	Built; add welfare check authorization
SteadyCare Non-Clinical Care Plan and Family Communication Plan	Step 9	Built
Care Coordination Tasks Log	Steps 9 to 10	Built
Transportation and Appointment Request	Step 10	Built
Incident Report; Client Behavior Documentation	Step 10	Built
Documentation Templates (STAR): telephonic, home visit, provider, Medication Concierge, SteadyCare	SOP-04 to 07, 12	In use
Monthly Provider Update	Step 10	To confirm
LCSW Consulting Agreement	Before first referral	Draft; legal review pending
RN Reviewer Consulting Agreement	Before first review	To build; legal review pending
Client Handbook (with client rights and complaint steps)	Step 8	Update for SOP-17

Appendix B: Quick Reference Card

Contact need	Number or resource
Emergency services	911
Suicide and Crisis Lifeline	988
Mississippi Adult Protective Services	1-844-437-6282 (hotline)
Poison Control	1-800-222-1222
SouthernWell main line	601-554-6067
Local law enforcement non-emergency (welfare checks)	[add number]

Contact need	Number or resource
Consulting LCSW	[name and number]
Consulting RN Reviewer	[name and number]
Backup contact (SOP-15)	[name and number]

Appendix C: Revision Log

Date	Summary of changes	Author / reviewer
September 2026	Initial SOP draft; service area and non-clinical wording updated; brand identity palette integrated.	Owner
October 2026 (Rev 2)	**Alignment:** service area corrected to Forrest, Lamar, Jones and Stone; programs and prices updated to Essentials, Signature, Concierge, add-ons and Telephonic Program; Senior Navigation removed; SteadyCare naming used; Rose Pink removed from palette. **Intake:** SOP-02 and SOP-03 rebuilt around the 10-step Workflow Manual, two gates, authority to sign, four-test fit decision, capacity cap and waitlist; "psychosocial intake" retired from SouthernWell scope. **Safety:** SOP-07 adds provider-directed parameters; SOP-10 defines missed check-in attempts and welfare check steps; SOP-05 adds lone-worker steps; SOP-06 adds exception handling. **Records and privacy:** same-business-day documentation; late entry rule; incident handling for privacy events; Release expiration tracking. **Business:** referral compensation rule (SOP-09, SOP-14); late payment procedure; continuity plan (SOP-15); intake performance measures (SOP-16). **New:** SOP-17 Client Rights, Concerns and Complaints; SOP-18 Discharge, Transition and Record Closure; Appendix D open items.	Owner (draft for review)

Appendix D: Open Items for Owner and Legal Decision

These items affect how this guide should read. Each needs a decision before the guide is finalized.

#	Item	Why it matters
1	Confirm the active caseload cap used in SOP-02 (currently the 15 to 20 client planning range).	Drives the Capacity test and the waitlist.
2	Legal review of the Community Referral Program, especially credits to social workers and healthcare professionals.	SOP-09 bars referral compensation with the LCSW; the program as written could conflict. Interim rule excludes licensed professionals.
3	Confirm Medication Concierge pricing: $25 add-on vs. $125 / $150 standalone plans.	The documents currently show both.
4	Approve the proposed late-payment timeline (SOP-14) and the cancellation and refund terms in the Service Agreement.	Needed for consistent, fair enforcement.
5	Record retention and destruction periods; client records request timeline; privacy incident notification duties.	Set by law and counsel; SOP-12 and SOP-13 refer to them.
6	Confirm the Mississippi reporting duty and APS hotline number with counsel or MDHS.	SOP-11 depends on it.
7	Confirm whether HIPAA applies to SouthernWell's activities and whether Business Associate or other agreements are needed with the LCSW and RN reviewer.	The HIPAA authorization and consulting agreements depend on the answer.
8	Confirm the missed check-in welfare check timing (three attempts, then 2 hours) and the use of local law enforcement for welfare checks.	SOP-10 sets a default; adjust to your clients and county.
9	Confirm program naming: SteadyCare, Steady Wellness™ and the retired "Steady Roots."	Names appear in forms, SOP-07 and the Price Sheet.

Appendix E: Document and Brand Standards

Apply the current SouthernWell palette to all operational binders, packets and digital documents.

Element	Hex	Use
Olive Green	#556B2F	Document titles, section and table headers
Sage	#8A9A86	Borders, dividers, alternating shading
Magnolia Cream	#FDFBF7	Page background
Sweetgrass	#9EAB84	Icons, divider rules, secondary subheadings
Deep Pecan	#4A3B32	Body text, callouts, emphasis and footer text (replaces Rose Pink)

Tagline: “Supporting Healing. Strengthening Families.” Use the open-door logo on covers. Header and footer text appear on every interior page.